Neutrophils	62.5	%	40-80	Double hydrodynamic sequential system/Microscopy
Lymphocytes	24.3	%	20-40	Flowcytometry DHSS/ Microscopy
Monocytes	9.5	%	2-10	Flowcytometry DHSS/ Microscopy
Eosinophils	3.2	%	1-6	Double hydrodynamic sequential system/Microscopy
Basophils	0.5	%	0-2	Double hydrodynamic sequential system/Microscopy
Absolute Leucocyte Count				
Absolute Neutrophil Count	3.77	10^3/µL	2-7	Calculated
Absolute Lymphocyte Count	1.47	10^3/µL	1-3	Calculated
Absolute Monocyte Count	0.57	10^3/µL	0.2-1	Calculated
Absolute Eosinophil Count	0.19	10^3/µL	0.02-0.5	Calculated
Absolute Basophil Count	0.03	10^3/µL	0.02-0.1	Calculated
Platelet Count	229	10^3/µL	150-410	Impedence Variation /Microscopy

NABL certificate and scope

This test has been performed at
TATA 1MG BANGALORE
Address: No 607, Ground, 1st,2nd, & 3rd Floor, 80 Feet Road, 6th Block, Koramangala, Bengaluru, 560095

Dr. Pritha Aggarwal
MBBS, MD (Pathologist)
Consultant Pathologist
Reg No: 20120000011

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PO No :PO1905788028-108

Name	: Mr.HARSH KATARIA	Client Name	: TATA 1MG BANGALORE
Age/Gender	: 25/Male	Registration Date	: 24/Mar/2024 09:26AM

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Comment:

- ESR provides an index of progress of the disease and is widely used as an indicator of inflammation, infection, trauma, or malignant diseases. Changes are more significant than a single abnormal test
- It is specifically indicated to monitor the course or response to the treatment of diseases like rheumatoid arthritis, tuberculosis bacterial endocarditis ,acute rheumatic fever ,Hodgkins disease,temporal arthritis , and systemic lupus erythematosis; and to diagnose and monitor giant cell arteritis and polymyalgia rheumatica.
- An elevated ESR may also be associated with many other conditions, including autoimmune disease, anemia, infection,malignancy,pregnancy, multiple myeloma, menstruation, and hypothyroidism.
- Although a normal ESR cannot be taken to exclude the presence of organic disease, its rate is dependent on various physiologic and pathologic factors.
- The most important component influencing ESR is the composition of plasma. High level of C-Reactive Protein, fibrinogen, haptoglobin, alpha-1antitrypsin, ceruloplasmin and immunoglobulins causes the elevation of Erythrocyte Sedimentation Rate.
- Drugs that may cause increase ESR levels include: dextran, methyldopa, oral contraceptives, penicillamine, procainamide, theophylline, and Vitamin A. Drugs that may cause decrease levels include: aspirin, cortisone, and quinine

"Test conducted on Whole Blood - EDTA "

NABL certificate and scope

This test has been performed at
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NABL certificate and scope

This test has been performed at

TATA 1MG BANGALORE

Address: No 607, Ground, 1st,2nd, & 3rd Floor, 80 Feet Road, 6th Block, Koramangala, Bengaluru, 560095

Dr. Vinisha Nahata
MBBS, DCP (Pathology)
Consultant Pathologist
Reg No: 108310

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- A Positive result indicates malarial infection. False Positives may be seen due to cross reactivity and persistence antigenemia.
- False negatives may be seen in patient's with very low parasitic index .

Malaria P.f/P.v Ag Test is an immunochromatographic assay for the differential detection between Plasmodium falciparum Histidine -Rich Protein-II (HRP-II) and pLDH (plasmodium lactate dehydrogenase) specific to Plasmodium vivax in human whole blood.

NABL certificate and scope

This test has been performed at

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Dr. Pritha Aggarwal
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like Anemia, Polycythemia.

NABL certificate and scope

This test has been performed at

TATA 1MG BANGALORE

Address: No 607, Ground, 1st,2nd, & 3rd Floor, 80 Feet Road, 6th Block, Koramangala, Bengaluru, 560095

Dr Ashwin Kumar A.S
MBBS M.D (Biochemistry)
Consultant Biochemist
Reg No:68123

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- It is a rapid, qualitative, screening test for early detection of antibodies to Salmonella Typhi in human serum/plasma. All positive results should be confirmed by supplement tests.
- A negative result does not rule out recent infection, as positive result is influenced by the time elapsed after the onset of fever and immuno- competence of the patient.

NABL certificate and scope

This test has been performed at

TATA 1MG BANGALORE

Address: No 607, Ground, 1st,2nd, & 3rd Floor, 80 Feet Road, 6th Block, Koramangala, Bengaluru, 560095

Dr. Vinisha Nahata
MBBS, DCP (Pathology)
Consultant Pathologist
Reg No: 108310

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- Widal test is a serological test, used for invitro detection and quantitative estimation of specific antibodies to Salmonella antigen (O, H, AH, & BH) in the serum.
- A positive Widal test confirms Enteric fever (typhoid fever or paratyphoid fever) caused by Gram negative bacteria, Salmonella enterica sub spp typhi or paratyphi A, B. However, A false positive result can sometimes be obtained by slide Widal test, which needs to be confirmed by Tube Widal test.
- The Widal test uses 'O' and 'H' antigens of S.typhi, S. paratyphi A and S. paratyphi B.
- Titers ≥ 1:80 of O antigen, H antigen of S. typhi S. paratyphi A & B are significant.
- In the case of Low titres, it is suggested to perform a repeat test after a week. A four fold rise in titre with gap of 1 week confirms the Widal test.

LIMITATION:

1. **False Positive:** Anamnestic response is seen in people who have had prior enteric infection or immunisation with TAB vaccine. This response is seen during an unrelated fever like- Malaria, Tuberculosis, Dengue, Influenza, Brucellosis, Rheumatic fever etc. A transient rise in H antibody titre is seen in such cases, whereas in the patients with enteric fever a sustained rise is observed.
2. **In endemic areas,** people may show moderately elevated levels of 'O' and 'H' agglutinins.
3. **False negative:** seen in early course of disease (1st week) and in immunosuppression.
4. **False negative** results can be seen in patient where antibiotic treatment is started before the sample is collected.

COMMENTS:

Widal Test detects antibodies against common somatic 'O' antigen and flagellar 'H' 'AH' 'BH' antigens of Salmonella typhi, paratyphi

NABL certificate and scope

This test has been performed at
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Address: No 607, Ground, 1st,2nd, & 3rd Floor, 80 Feet Road, 6th Block, Koramangala, Bengaluru, 560095

Dr. Vinisha Nahata
MBBS, DCP (Pathology)
Consultant Pathologist
Reg No: 108310

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1. This is only a screening test . All samples detected reactive must be confirmed by using confirmatory test.
2. False positive results can be obtained due to cross reaction with Epstein-BARR virus, Influenza A & B, Brucella and Dengue virus.
3. False negative results are seen if IgM antibody is below the detectable limit.
4. A negative result does not preclude the possibility of exposure or infection with CHIKV

NABL certificate and scope

This test has been performed at

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Note:

- The referring centre/ Lab is responsible for informing concerned Local authorities on notifiable disease.
- Recommended test is NS1 Antigen by ELISA in the first 5 days of fever. After 7-10 days of fever, the recommended test is Dengue virus antibodies IgG & IgM by ELISA.
- NS1 Positive diagnosis to be confirmed by IgM Capture ELISA.

Comments:
Dengue viruses belong to the family Flaviviridae and have 4 serotypes (1-4).It is transmitted by the mosquito Aedes aegypti and Aedes albopictus and is widely distributed in Tropical and Subtropical areas of the world. The disease may be subclinical, self limiting, febrile or may progress to a severe form of Dengue hemorrhagic fever or Dengue shock syndrome.

Positive: The presence of Dengue nonstructural protein 1 (NS1) antigen is typically detectable within 1 to 2 days following infection and up to 9 days following symptom onset.NS1 antigen may also be detectable during secondary dengue virus infection, but for a shorter duration of time (1-4 days following symptom onset).

Negative: The absence of dengue NS1 antigen is suggestive of absence of acute phase of the infection. The NS1 antigen may be negative if specimen is collected too early such as immediately following dengue virus infection (<24-48 hours) or is collected following 9 to 10 days of symptoms. Results should always be interpreted in conjunction with clinical presentation and exposure history.

Limitations:

- Cross reactivity is seen in the Flavivirus group between Dengue virus,Zika virus, Murray Valley encephalitis, Japanese encephalitis, Yellow fever & West Nile viruses.
- Negative NS1 antigen results may occur if the specimen was collected after 7 days following symptom onset. Serologic testing for the presence of IgM and IgG antibodies to Dengue Virus is recommended in such cases.

NABL certificate and scope

This test has been performed at
TATA 1MG BANGALORE
Address: No 607, Ground, 1st,2nd, & 3rd Floor, 80 Feet Road, 6th Block, Koramangala, Bengaluru, 560095

Dr. Trupthi Gowda
MBBS, M.D (Microbiology)
Consultant Microbiologist
Reg No: 87170

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Bilirubin	NEGATIVE		Negative	Diazonium
Urobilinogen	NORMAL		Normal	Ehrlich
Leucocyte Esterase	NEGATIVE		Negative	Pyrrole
Nitrite	NEGATIVE		Negative	Diazonium Compound
Pus cells	2-3	/hpf	0-5	Microscopy
Red Blood Cells	NIL	/hpf	0-2	Microscopy
Epithelial cells	1-2	/hpf	Few	Microscopy
Casts	NIL	/lpf	Nil	Microscopy
Crystals	NIL		Nil	Microscopy
Yeast	NIL		Nil	Microscopy
Bacteria	NIL		Nil	Microscopy

Comment:

•Note: Pre-test condition to be observed while submitting the sample-first void, mid stream urine, collected in a clean, dry, sterile container is recommended for routine urine analysis, avoid contamination with any discharge from vaginal, urethra, perineum, Avoid prolonged transit time & undue exposure to sunlight.

•During interpretation, points to be considered are Negative nitrite test does not exclude the urinary tract infections. Trace proteinuria can be seen with many physiological conditions like prolonged recumbency, exercise, high protein diet. False positive reactions for bile pigments, proteins, glucose and nitrites can be caused by peroxidase like activity by disinfectants, therapeutic dyes, ascorbic acid and certain drugs.• Urine microscopy is done in centrifuged urine specimens

***** End Of Report *****

Conditions of Laboratory Testing & Reporting:

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